BY ORDER OF THE COMMANDER
59TH MEDICAL WING

59TH MEDICAL WING INSTRUCTION
46-104

7 FEBRUARY 2023

Nursing

NURSING CARE

COMPLIANCE WITH THIS PUBLICATION IS MANDATORY

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RELEASABILITY: There are no releasability restrictions on this publication.

OPR: 59 MDW/SGN

Certified by: 59 MDW/SGN
(Colonel Paul Miller)

Supersedes: 59 MDWI 46-104, 6 April 2021

Pages: 7

This instruction supplements Air Force Policy Directive 46-1, *Nursing Services.* This instruction describes the plan for providing nursing care based on the wing mission and needs of the patient population. It defines nursing, nursing standards, identifies areas in the facility/unit where nursing care is delivered, and the responsibilities of the 59th Medical Wing (MDW) Chief Nurse (SGN) and Group SGN's. It establishes the standard for demonstrating the nursing process. This instruction applies to all nursing personnel assigned, attached or under contract to the 59 MDW. It excludes 959th Medical Group and does apply to the Air National Guard or Air Force Reserve if they are working in the 59th MDW. Refer recommended changes and questions about this publication to the Office of Primary Responsibility using the AF Form 847, *Recommendation for Change of Publication*. The authority to waive requirements is the publication approval authority. Ensure that all records created as a result of processes prescribed in this publication are maintained IAW AFI 33-322, *Records Information Governance Program*, and disposed of IAW Air Force Records Information Management System (AFRIMS) Records Disposition Schedule (RDS).

SUMMARY OF CHANGES

This publication has been revised. This rewrite of 59 MDWI 46-104 includes updated areas where care is delivered.

1. General.

1.1. Nursing services has adopted the Defense Health Agency's (DHA) vision of Ready, Reliable Care, a continuous learning and improving organization with a single-minded focus of patient safety and Zero Harm.

1.2. Nursing personnel are directly responsible for ensuring patients receive nursing care based on the nursing process. Continuity and communication of nursing care is achieved whenever the care of a patient is transferred from one caregiver to another, and is documented in the patient's medical record. Nursing personnel includes all active duty, (Registered Nurses /medical technicians), General Schedule and contract employee Registered Nurses /Licensed Vocational Nurses/Licensed Practical Nurses/students/AD/Guard doing time.

2. Overview.

2.1. Nursing. The 59 MDW has adopted the American Nursing Association (ANA) definition of professional nursing as the protection, promotion, and optimization of health and abilities, prevention of illness and injury, facilitation of healing, alleviation of suffering through the diagnosis and treatment of human response, and advocacy in the care of individuals, families, groups, communities, and populations. There are six essential features of professional nursing:

2.1.1. Provision of a caring relationship that facilitates health and healing.

2.1.2. Attention to the range of human experiences and responses to health and illness within the physical and social environments.

2.1.3. Integration of objective data with knowledge gained from an appreciation of the patient's or group's subjective experience.

2.1.4. Application of scientific knowledge to the processes of diagnosis and treatment through the use of judgment and critical thinking.

2.1.5. Advancement of professional nursing knowledge through evidence-based practice.

2.1.6. Influence on social and public policy to promote social justice.

2.2. This definition of nursing reinforces utilization of the nursing process, including data collection, diagnosis, planning, treatment and evaluation. The nursing process is supported by standards of nursing practice that are congruent with the definition and that provide more specific guidelines for practice. These standards include systematic, continuous collection of data concerning the health status of the patient in recorded form that is accessible and that may be communicated. A nursing diagnosis is derived from the data collected. A plan for nursing care incorporates goals derived from the nursing diagnosis and the priorities and approaches to achieve the goals as indicated by the nursing diagnosis.

2.3. Nursing Practice. The act of applying the knowledge of human responses to actual or potential health problems.

2.4. Nursing Process. A methodology that facilitates nurses' identification of human responses to health problems. The framework used within the 59 MDW is one adopted from the ANA's Standards of Clinical Nursing Practice.

3. Additional Standards.

3.1. The Nursing Executive Committee (NEC) may add standards, as necessary. These standards may be outcome, process, procedural, or structural in nature. Individual units and clinics may also adopt professional practice standards as needed.

3.2. 59 MDW is located in the State of Texas. As such, the Texas Nursing Practice Act **http://www.bon.texas.gov/practice_scope_of_practice_rn.asp** is utilized in assessing Standards of Care.

4. Responsibilities of the 59 MDW/SGN (Chief Nurse).

4.1. The 59 MDW/SGN is responsible and accountable for ensuring that the provision of patient care and standards of nursing practice are consistent with current evidence-based research and nationally recognized professional standards. The SGN provides wing oversight in the development of nursing standards of practice, policy, and resource requirements.

4.2. The 59 MDW/SGN and 59 MDW 4N Functional Manager have the authority to define, establish, and implement nursing standards for patient care and nursing practice throughout the 59 MDW.

4.3. The 59 MDW/SGN chairs the NEC, which is responsible for providing oversight for the provision of nursing care within the 59 MDW, establishing nursing policy and standards of practice, and for collaborating, planning, coordinating, designing and integrating the delivery of nursing care, services, and research within the 59 MDW.

4.4. Provision of Nursing Care. The 59 MDW/SGN/4N Functional Manager, in conjunction with the Group SGNs/4N Functional Managers, ensure appropriate nursing support for medical services. When plans for change in the provision of specific patient care services are developed, they apply appropriate changes to nursing care requirements and resource projections.

4.5. The ongoing provision of nursing care is monitored through the review of performance improvement activities, peer review, customer satisfaction data tracked through the Patient Advocacy Office, patient comment cards and letters, random patient medical record reviews, and other means. The 59 MDW/SGN informs the Board of Directors (BOD) when potential compromises in patient care or practice patterns could occur due to inadequate numbers of qualified nursing personnel.

4.6. When action is required to modify practice or address staffing and training issues that have actual or potential impact on nursing care, the 59 MDW/SGN takes action through the NEC, Performance Improvement Committee, Healthcare Plans Management Committee and/or BOD.

4.7. Matters involving substandard clinical performance on the part of non-privileged nursing personnel are addressed with the Risk Management Office and acted on IAW guidelines in AFI 44-119, *Medical Quality Operations*.

5. Responsibilities of the Group SGN.

5.1. The Group SGN is responsible and accountable for ensuring that the provision of patient care and standards of nursing practice are consistent with current evidence-based findings and nationally recognized professional standards.

5.2. The Group SGNs/4N Functional Managers collaborate with the 59 MDW/SGN/4N Functional Manager to ensure nursing standards are upheld within their groups.

5.3. Group SGNs/4N Functional Managers ensure appropriate nursing support is available to provide medical care. When changes to staffing are required, consideration is given to skill mix of nurses and medical technicians to ensure patient safety remains top priority. Group SGNs will inform the 59 MDW/SGN/4N Functional Manager of inadequate numbers of qualified nursing personnel.

5.4. Group SGNs participate in continuous process improvements to ensure processes are in place for all members of the group to achieve zero harm to patients.

5.5. Group SGNs continuously monitor performance improvement activities. They will inform the NEC and group executive committee members when action is required to modify practice or there is a need to address staffing or training issues impacting patient care.

5.6. Group SGNs participate in, and foster participation by nursing personnel, in the Evidence Based Practice Council which is a subset of the NEC. The Evidence Based Practice Council purpose is to foster evidence-based practice within the 59 MDW by engaging nursing personnel to use interdisciplinary methodology to seek out and translate current evidence-based research to advance nursing practice and optimize patient outcomes.

5.7. Substandard clinical performance is addressed with the Risk Management Office IAW DHA-PM 6025.13.

6. Quality of Nursing Care.

6.1. Quality of nursing care is monitored by the 59 MDW and Group SGNs and 4N Functional Managers through several sources.

6.1.1. Direct and indirect observation of staff performance by group chief nurses, 4N Functional Managers, superintendents, flight chiefs/noncommissioned officers in charge.

6.1.2. Nursing Peer Review Process. (The peer review process and management of clinical adverse actions are governed IAW DHA-PM 6025.13, *Implementation Guidance for Defense Health Agency Procedures Manual, "Clinical Quality Management in the Military Health System,".*

6.1.3. Wing Risk Management input addressing practice concerns/trends.

6.1.4. Information provided from Patient Safety, Infection Control, and from other health care disciplines addressing the quality of nursing care and from other services.

6.1.5. Patient and customer feedback regarding nursing services provided at the 59 MDW.

7. Areas Where Nursing Care is Delivered.

7.1. The 59 MDW is comprised of six medical groups. A Chief Nurse is embedded at the following locations: 959 Medical Group, Brooke Army Medical Center, Fort Sam Houston; 59 Medical Operations Group, Wilford Hall, JBSA-Lackland; 559 Medical Group, Reid Clinic, JBSA-Lackland; 559 Medical Squadron, JBSA-Randolph and at 59 Training Group, Medical Education and Training Campus (METC), Fort Sam Houston. The Dental Group and Medical Support Group (MDSG) do not have a SGN. Each group has a unique mission which nursing personnel support as follows:

7.2. 59 Medical Operations Group. 59 MDOG nursing personnel deliver urgent, surgical, medical specialty and primary care in the DoD's largest ambulatory surgical center. 59 MDOG includes the AFMS's largest Family Medicine clinic, largest Internal Medicine clinic, largest Pediatric clinic, a Level IV Family Emergency Center with the DoD's busiest Ambulance Service section embedded within, as well as 4 operating rooms and 39 other clinical specialty areas.

7.3. 59 Training Group (TRG). Provides training for nurses and 17 technical AFSCs. The 59 TRG ensures national standards and maintains 14 program accreditations as well as directs nursing education and readiness skill development for 7.3K trainees. In addition, they manage the Nursing personnel and provide training for seven professional nursing programs which include the Air Force Perioperative Nursing Course, Perinatal Nursing Course, Critical Care/Emergency Trauma Nursing Course, Psychiatric Mental Health Nursing Course, Neonatal Intensive Care Nursing Course, Nurse Transition Program and the Nurse Residency Program course administration at 10 military and 4 civilian clinical training sites across the United States. Further, they provide the supplementary courses of Air Force Medical Home Operations, Disease Management, Epidemiology, Prevention, and Infection Control, Health Care Integration, Health Professions Education and Training, and Nursing Service Management. The 59 TRG ensures that all these programs meet accreditation and national standards which enables safe and reliable care and the success of our mission.

7.4. 559 Medical Group. Nursing personnel delivers care for the AF's largest Training Wing, including 6 wings, 12 GSUs, and 122 mission partners. The broad spectrum of patients includes Basic Military Trainees, Technical Training students, Aircrew, Special Warfare Trainees, Defense Language Institute students, and beneficiaries. Nursing personnel fill positions in the Trainee Health, Flight and Operational Medicine, Occupational Medicine, Sports Medicine, and Base Operational and Medicine clinics. Personnel also supports the En-Route Patient Staging System, providing care to approximately 700-1K Aeromedical Evacuation patients yearly.

7.5. 559 Medical Squadron. Is located on Randolph Air Force Base and is led by the 559 Medical Group at Lackland Air Force Base. Provides care to personnel in the 12th Flying Training Wing, the Air Force Personnel Center, Air Education and Training Command, as well as additional 30 mission partners and their beneficiaries. Nursing personnel fill positions in four outpatient clinics to include Family Practice, Pediatrics, Flight Medicine, and Warfighter Operational Medicine Clinic. Additionally, nurses fill positions in the Family Advocacy Program, Behavioral Health Outpatient Program and Medical Management.

7.6. 59th Dental Group. Two perioperative nurses provide care during dental surgical cases.

7.7. 59 MDW, Wing Staff:

7.7.1. Science and Technology (ST). Three Nurse Scientists lead nursing research within the Nursing Research Division of ST.

7.7.2. Division of Wing Education and Training (DET). Conducts and oversees training for the 59 Medical Wing. DET Personnel validate requirements and train Wilford Hall Ambulatory Surgical Center personnel in Basic Life Support, Advanced Cardiac Life Support, Pediatric Advanced Life Support, National Registry of Emergency Medical Technicians and train the trainer Self-Aid and Buddy Care instructors courses. They

maintain and update Elsevier Healthcare data, oversee the Independent Medical Technician Program; instruct Joint Service Trauma Course and the Ability to Survive and Operate Course. Conduct Competency Assessment Folders maintenance training for unit Education and Training representatives. Lastly, the DET coordinates and conducts monthly The Joint Commission Newcomers Briefs, creates and maintain Training Affiliation Agreements, coordinates JCCQAS and gives oversight to Unit Training Managers and establishes training and provides guidance on all enlisted Air Force training records, ancillary training and provides reports to leadership.

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Director, Wilford Hall Ambulatory Surgical Center

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

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Adopted Form

AF Form 847, *Recommendation for Change of Publication*

Abbreviations and Acronyms

AFMS—Air Force Medical Service

ANA—American Nursing Association

BOD—Board of Directors

DET—Division of Wing Education and Training

DHA—Defense Health Agency

IAW—In Accordance With

MDSG—Medical Support Group

MDW—Medical Wing

NEC—Nursing Executive Committee

SGN—Chief Nurse

ST—Science and Technology

TRG—Training Group